



HOSPITAL SERIAN

TITLE: POLICY FOR OUTBREAK

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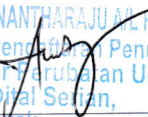
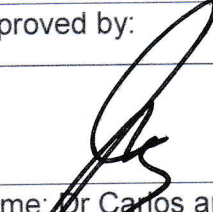
UNIT/DEPARTMENT/DIVISION: Outbreak Policy of HOSPITAL SERIAN

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POLICY FOR OUTBREAK

Policy Statement

Outbreaks and incidents of infection have serious consequences for service users including mortality, morbidity, distress, delays in treatment and impacts on service provision. This policy is primarily intended to cover outbreaks involving staff and service users at Hospital Serian.

1. Introduction

Definition of an outbreak and outbreak control team.

An outbreak may be defined as:

- an incident in which 2 or more people experiencing a similar illness are linked in time within locality and department. (Communicable Disease and MDRO)
- a greater than expected rate of infection compared with the usual background rate for the place and time where the outbreak has occurred.
- a single case for certain rare or high-consequence diseases such as diphtheria, botulism, rabies, viral haemorrhagic fever or polio.

It is important that such cases are appropriately recorded and managed for audit purposes and to support surveillance and any future outbreak management. In some instances, more routine or low-risk outbreaks may be managed through standard operating procedures, with the option to escalate should the risk assessment change.

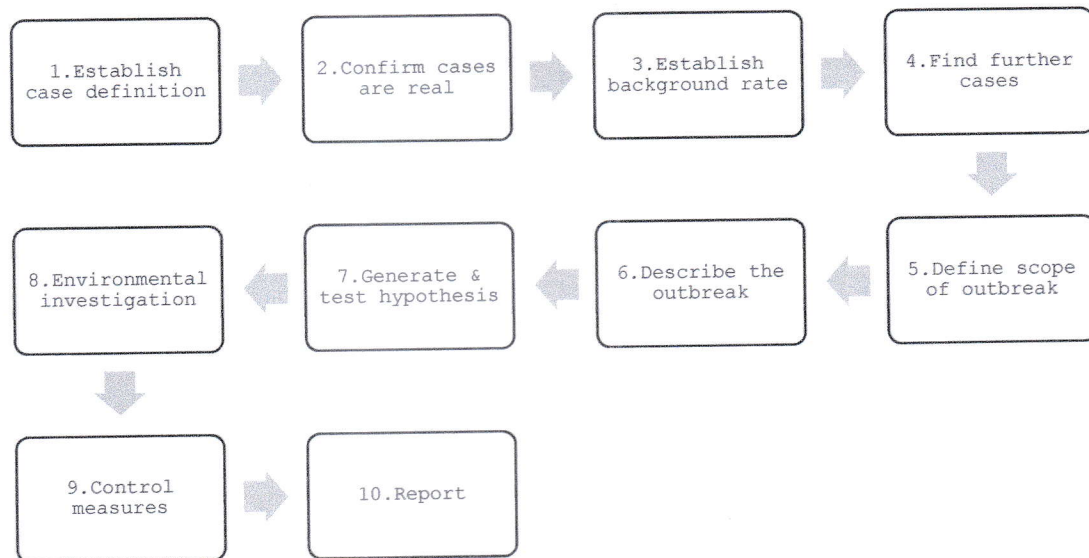
The primary objective in outbreak management is to protect public health by identifying the source and cause of infection and transmission dynamics, to implement control measures to prevent further spread or recurrence.

Secondary objectives include refining outbreak management, adding to the evidence base about sources and transmission of infectious agents and control measures, training and lessons identified for improving communicable disease control.

2. Principles of Outbreak Investigations and Management

- 2.1 Establish case definition
- 2.2 Confirm cases are real
- 2.3 Establish background rate
- 2.4 Find further cases
 - 2.4.1 Find further cases with similar infection
 - 2.4.2 Link to time, place and person
 - 2.4.3 Identify contacts- map the patient movement and screen contact
- 2.5 Define scope of outbreak
 - 2.5.1 Major or minor
- 2.6 Describe the outbreak
 - 2.6.1 Outbreak data
 - 2.6.2 Formulate a hypothesis
 - 2.6.3 Identify source & route of infection
 - 2.6.4 Suggest & implement initial control measures
- 2.7 Generate & test hypothesis
- 2.8 Environmental investigation
 - 2.8.1 Collect samples & directly observe care
- 2.9 Control measures
- 2.10 Report
 - 2.10.1 Describe the outbreak
 - 2.10.2 Remedial measures implemented
 - 2.10.3 Effectiveness of remedial measures
 - 2.10.4 Recommendation to prevent future recurrences

3. Flowchart outbreak management.



4. Fundamental Aspects Management of MDRO Infections

- 4.1 Infection control measures requirement
 - 4.1.1 Standard & Contact
- 4.2 Personal safety/protections:
 - 4.2.1 Personal Protective Equipment
 - 4.2.2 Hand Hygiene
- 4.3 Minimising exposure:
 - 4.3.1 Patients' Flow & Movements
 - 4.3.2 Patient's Placement
- 4.4 Healthcare worker Management
 - 4.4.1 Limit number of HCWs and support staff in contact with a patient probable with confirmed infection
 - 4.4.2 Strict hand hygiene
 - 4.4.3 Education
 - 4.4.4 Emphasis on complete PPE
- 4.5 Infection control precautions when caring for patients: Family and visitors
 - 4.5.1 Should be limited to those essential for patient support and care
 - 4.5.2 Should be educated on the risk of transmission and barrier precautions especially on the importance of hand hygiene
 - 4.5.3 Family members involved with direct care of the patient should use similar PPE as required for the HCW
 - 4.5.4 They are discouraged from bringing unnecessary personal items. Items in contact with patients should be cleaned before leaving the room